

Office of Enforcement and Removal Operations  
ICE Health Service Corps

U.S. Department of Homeland Security  
500 12th Street, SW  
Washington, D.C. 20536



U.S. Immigration  
and Customs  
Enforcement

February 5, 2026

MEMORANDUM FOR:

(b)(6),(b)(7)(C),(b)(7)(F), DHSc, FACHE  
Assistant Director  
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

CAPT (b)(6),(b)(7)(C),(b)(7)(F) MD, MPH, CCHP, FAAFP  
Chief Medical Officer  
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CDR (b)(6),(b)(7)(C),(b)(7)(F) RN  
Chief, Investigations Unit  
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FROM:

CAPT (b)(6),(b)(7)(C),(b)(7)(F) RN  
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LCDR (b)(6),(b)(7)(C),(b)(7)(F) RN  
Senior Investigator, Investigations Unit  
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SUBJECT:

Mortality Review - Report of Findings  
Ismael AYALA-Urbe, (b)(6),(b)(7)(C)

**Executive Summary:**

On September 22, 2025, the U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of alien Ismael AYALA-Urbe, (b)(6),(b)(7)(C). ICE detained Mr. AYALA-Urbe, a 39-year-old Mexican male, at Adelanto ICE Processing

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Center (AIPC), in Los Angeles, California, from August 22 to September 22, 2025. Mr. AYALA-Urbe's autopsy is pending.

Prior to his detention, on an unknown date, Mr. AYALA-Urbe presented to the United States at an unknown place without being admitted or paroled by an immigration officer. On October 5, 2012, Mr. AYALA-Urbe filed for Consideration of Deferred Action for Childhood Arrivals (DACA), and the U.S. Citizenship and Immigration Services (USCIS) approved his application with extensions through July 11, 2014. Between May and September of 2015, law enforcement arrested Mr. AYALA-Urbe for driving under the influence (DUI) of alcohol and sentenced him to three years' probation. On October 4, 2016, USCIS denied his DACA renewal application. On May 23, 2018, law enforcement arrested him again for DUI and sentenced him to 120 days in jail and five years' probation upon conviction. On August 17, 2025, the U.S. Border Patrol arrested Mr. AYALA-Urbe for expired employment authorization documentation and denied DACA, and transferred him to ICE custody. On August 22, 2025, ICE detained him at AIPC.

On August 22, 2025, during his AIPC intake screening, Mr. AYALA-Urbe denied a medical or mental health history, and medication use.

On September 18, 2025, medical staff responded to a medical emergency in Mr. AYALA-Urbe's dorm for his complaint of buttock pain. A registered nurse (RN) evaluated him and contacted a physician, and the physician issued verbal orders to the medical staff for medications and twice daily wellness checks. The nursing staff administered pain medication to Mr. AYALA-Urbe, and he reported feeling better and returned to his dormitory. After his evaluation, Mr. AYALA-Urbe continued to receive his medications.

On September 21, 2025, custody staff sent Mr. AYALA-Urbe to the medical clinic due to his ill appearance and complaints of discomfort. Medical staff contacted the on-call physician due to his abnormal symptoms and vital signs, and the physician ordered his transfer to Victor Valley Global Medical Center (VVGMC), by ambulance, for emergency evaluation for possible incision and drainage.

On September 22, 2025, VVGMC medical staff reported Mr. AYALA-Urbe experienced hypotension and tachycardia and went into respiratory arrest. Medical staff performed cardiopulmonary resuscitation (CPR), achieved return of spontaneous circulation, intubated him, and moved him to the intensive care unit. At approximately 2:00 a.m., Mr. AYALA-Urbe suffered cardiac arrest, and hospital staff were unable to revive him. At 2:32 a.m., a physician pronounced Mr. AYALA-Urbe deceased.

### **Mortality Finding:**

As a result of Mr. AYALA-Urbe's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up

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to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues requiring further study.

A mortality review committee (MRC) determined the care AIPC provided to Mr. AYALA-Urbe deviated beyond safe limits of practice. Specifically, the MRC determined, 1) Medical staff did not thoroughly evaluate Mr. AYALA-Urbe or complete regular wellness checks. 2) Nursing staff did not adequately relay Mr. AYALA-Urbe's status to the on-call provider. 3) Medical staff did not effectively transfer care from one staff member to another. 4) Medical staff did not consistently respond to Mr. AYALA-Urbe's medical complaints timely. 5) Medical staff did not administer all of Mr. AYALA-Urbe's pill line medications. 6) Medical staff did not adequately document all of Mr. AYALA-Urbe's care timely, all discussions related to his plan of care, or his refusal of ordered medications. 7) AIPC's staffing hindered their ability to provide their patient population medical care in accordance with timeframes established by accreditations standards.

The MRC convened on November 18, 2025, and consisted of the following IHSC members: Dr. (b)(6),(b)(7)(C),(b)(7)(F), medical doctor, clinical director, physician reviewer; Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) physician assistant, IHSC chief advanced practice provider (APP), APP reviewer; and Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) RN, central regional nurse manager, nurse reviewer.

The following summary of events was based on: 1) Mr. AYALA-Urbe's AIPC medical records; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. AYALA-Urbe's AIPC detention file; 5) video footage from September 18, 2025; 6) an on-site review and staff interviews conducted by CAPT (b)(6),(b)(7)(C),(b)(7)(F) and Lieutenant Commander (LCDR) (b)(6),(b)(7)(C),(b)(7)(F) at AIPC, from October 28 to 30, 2025; 8) applicable AIPC policies and procedures; and 9) applicable accreditation standards.

### ***Summary of Events***

On August 22, 2025, an AIPC licensed practical nurse (LPN) completed Mr. AYALA-Urbe's medical intake screening, noted his denial of chronic health conditions or medication use, documented a normal mental health screening, physical exam, and vital signs, except for a slightly elevated blood pressure (BP) of 137/79 millimeters of mercury [(mmHg); normal: between 90/60 and 120/80 mmHg] and high body mass index of 46.47 (severe obesity), and cleared him for general population.

On September 17, 2025, Mr. AYALA-Urbe submitted a medical request for health services for an anal abscess causing him pain. An RN documented receipt of the request on September 19, 2025, and wrote "midlevel appointment upcoming" in the comments section for action taken.

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On September 18, 2025, at 2:06 p.m., medical staff responded to a medical emergency in Mr. AYALA-Urbe's dorm for complaints of buttock pain, transported him to the medical clinic in a wheelchair, and documented he reported a loose last bowel movement and no history of hemorrhoids. An RN documented sharp anal pain, rated 10/10 on a 10-point pain scale in which 10 is severe, and the following vital signs: BP of 168/87 mmHg, temperature of 99.3 degrees Fahrenheit [(°F); normal: 97.6°F – 99.6°F], heart rate of 103 beats per minute [(BPM); normal: 60 – 99 BPM], respirations of 18 breaths per minute (normal: 12 – 20 breaths per minute), and oxygen (O<sub>2</sub>) saturation of 93% (normal: greater than 95%). The RN contacted a physician, who spoke with Mr. AYALA-Urbe by phone to assess his condition. An LPN documented verbal orders for wellness checks twice daily for ten days and the following medications: acetaminophen (for pain) 500 milligrams (mg), two tablets, orally, three times a day; hemorrhoidal ointment (for rectal pain and inflammation), rectally, once a day; ibuprofen (pain and inflammation) 800 mg, one tablet, orally, three times a day as needed; ketorolac (for pain) 30 mg per milliliter (mg/mL), inject 30 mg in the right arm and 30 mg in the left arm, intramuscularly, now; and psyllium/aspartame (laxative) one packet dissolved in water, orally, twice daily. Medical staff immediately administered Mr. AYALA-Urbe's pain medication in the clinic and allowed him to return to his dorm when he reported feeling better.

On September 20, 2025, at 11:36 a.m., custody staff called medical staff to request a wellness check on Mr. AYALA-Urbe due to his ill appearance, but medical staff did not return the officer's call or follow up on the request.

- At 11:15 p.m., custody staff called the medical clinic, requesting medical staff evaluate Mr. AYALA-Urbe due to his ill appearance. A medical staff member informed the officer all medical staff were preoccupied with other emergencies, and medical staff were waiting on a custody officer who could bring Mr. AYALA-Urbe to the medical clinic.

On September 21, 2025, at 12:56 a.m., custody staff transported Mr. AYALA-Urbe to the medical clinic. An RN evaluated Mr. AYALA-Urbe for his complaint of 10/10 pain and a "lump" on his buttock area, and 8/10 epigastric (upper abdomen) pain. The RN documented Mr. AYALA-Urbe appeared diaphoretic and shaky, reported feeling nauseous, had vomited once, and had an elevated heart rate of 160 BPM, a respiration rate of 25 breaths per minute, BP of 104/65 mmHg, O<sub>2</sub> saturation of 93% on room air, and a blood glucose reading of 161 milligrams per deciliter [(mg/dL); normal: 70 – 90 mg/dL]. The RN completed an electrocardiogram, which showed abnormal results (sinus tachycardia, with a ventricular rate of 163 BPM, right bundle branch block, left posterior fascicular block, bifascicular block, inferior infarct), contacted the on-call physician, and received an order to send Mr. AYALA-Urbe to the hospital by ambulance for further evaluation and management.

- At 2:15 a.m., emergency medical services personnel transferred him to VVGMC.

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- At 11:04 a.m., VVGMC admitted Mr. AYALA-Urbe for acute pneumoperitoneum (presence of air in the peritoneal cavity that requires urgent medical evaluation and intervention), acute kidney disease (sudden loss of kidney function), systemic inflammatory response syndrome, and sepsis (life-threatening response to an infection).
- At 3:55 p.m., VVGMC medical staff reported Mr. AYALA-Urbe had been admitted to a surgical floor, appeared calm and cooperative, had a diagnosis of hypotension (low BP) and an abscess to his left buttock, underwent an ultrasound (results pending), and might undergo surgery to drain his abscess after stabilization of his BP. Medical staff reported Mr. AYALA-Urbe received intravenous fluids, rated his rectal pain at 10/10, and had an elevated heart rate of 113 BPM.

On September 22, 2025, at approximately 1:49 a.m., Mr. AYALA-Urbe experienced hypotension and tachycardia, and went into respiratory arrest. VVGMC staff performed CPR, achieved return of spontaneous circulation, intubated him, and moved him to the intensive care unit to start norepinephrine (raises blood pressure).

- At approximately 2:00 a.m., Mr. AYALA-Urbe suffered cardiac arrest. Hospital staff performed CPR; however, they were unable to revive him.
- At 2:32 a.m., a physician pronounced Mr. AYALA-Urbe deceased.

Mr. AYALA-Urbe's autopsy report is pending.

The MRC used the following accreditation standards for this review: Performance-Based National Detention Standards, 2011 with 2016 revisions, and National Commission on Correctional Health Care (NCCHC) Standards for Health Services in Jails, 2018. The following are identified program strengths, weaknesses, and recommendations found during this review.

### ***Strengths***

The MRC identified the following strengths: On September 18 and 20, 2025, AIPC custody staff recognized and communicated Mr. AYALA-Urbe's ill appearance to medical staff.

### ***Weaknesses***

#### **1. Inappropriate medical care.**

- On September 18, 2025, medical staff did not perform a visual assessment of Mr. AYALA-Urbe's buttock (area of complaint) during his medical emergency.
- From September 18, 2025, after the medical emergency visit, to September 21, 2025, nursing staff did not perform the twelve-hour wellness checks the physician ordered.

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Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section B. *Designation of Authority*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section A. *General*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section II. *Expected Outcomes*.
- AIPC Policy: 608-A, *Medical and Mental Health Services, Emergency Services*.

## **2. Communication.**

- Nursing staff did not relay Mr. AYALA-Uribe's abnormal vital signs to a higher-level provider during the intake screening on August 22, 2025, or during his medical emergency evaluation on September 18, 2025.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. *Emergency Medical Services and First Aid*.
- NCCHC Standards of Health Services in Jails, 2018; J-E-08. *Nursing Assessment Protocols and Procedures*.
- AIPC Policy: 608-A, *Medical and Mental Health Services, Emergency Services*.

## **3. Continuity of Care.**

- On September 18, 2025, an RN ineffectively transferred care to another RN for Mr. AYALA-Uribe during his medical emergency, causing a breakdown in communication regarding his care.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section Z. *Continuity of Care*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. *Emergency Medical Services and First Aid*.
- NCCHC Standards of Health Services in Jails, 2018; J-E-08. *Nursing Assessment Protocols and Procedures*.
- AIPC Policy: 608-A, *Medical and Mental Health Services, Emergency Services*.

## **4. Untimely response to medical complaints.**

- On September 17, 2025, Mr. AYALA-Uribe submitted a request for medical attention, and AIPC nursing staff did not triage the complaint within 24 hours.

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- On September 20, 2025, at 11:36 a.m., custody staff requested medical staff perform a wellness check on Mr. AYALA-Urbe due to his ill appearance, and medical staff did not respond to the request until the evening.
- On September 20, 2025, at 11:15 p.m., medical staff advised custody to bring Mr. AYALA-Urbe to the medical clinic for evaluation, and due to multiple emergencies and (b)(7)(E) Mr. AYALA-Urbe did not receive a medical evaluation until 12:56 a.m. on September 21, 2025.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section S. *Sick Call*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section Z. *Continuity of Care*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care, V. Expected Practices; A: *General*.
- AIPC Policy: 608-A, Medical and Mental Health Services, *Emergency Services*.
- AIPC Policy: 625-A, Medical and Behavioral Health Services, *Sick Call and Triage of Non-Emergent Requests for Health Services*.

## 5. Medication Administration.

- On September 19 and 20, 2025, nursing staff did not administer Mr. AYALA-Urbe's third dose of acetaminophen or second dose of psyllium or complete refusal forms to document his refusal of the scheduled medications.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section U. *Delivery of Medication*.
- AIPC Policy: 710-A, Medication Management, *Medication Administration*.

## 6. Documentation.

- On September 18, 2025, an RN responded to Mr. AYALA-Urbe's medical emergency, evaluated him, and contacted the on-call physician; however, the RN did not document this encounter until four days later and completed a late entry that lacked details of her report of Mr. AYALA-Urbe's vital signs to the physician or the conversation between the physician and Mr. AYALA-Urbe.
- On September 19 and 20, 2025, AIPC nursing staff documented Mr. AYALA-Urbe refused medications but did not complete a refusal form for the missed doses.

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Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section BB. *Medical Records*.
- GEO Plan 608-A: Medical and Mental Health Services, *Emergency Services*.
- AIPC Policy: 608-A, *Emergency Plan*.

## **7. Staffing.**

- From June to September 2025, AIPC experienced a rapid influx of patients due to recently lifted COVID restrictions. As a result, medical staff did not consistently provide care within time frames established by accreditation standards.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; Section B. *Designation of Authority*.

## ***Recommendations***

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Care Compliance Division (HCCD).
- The IHSC DAD of HCCD will share these findings through appropriate communication channels to ICE, and AIPC's facility administrator and health authority, for review and any action(s) deemed appropriate.
- The respective IHSC HCCD unit and ICE will ensure a corrective action plan, if applicable, is implemented and sustained.

End of report.